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/ **Practice Exam - 1**

Correct Answer

Next Q

Practice Exam - 1

Question 55 of 117



A 56-year-old woman presents to clinic for scheduled routine follow up. She has a five-year history of nonischemic dilated HFrEF (25-30%), diabetes mellitus, hypertension, hyperlipidemia and paroxysmal atrial fibrillation. She has been on stable doses of metoprolol succinate 150mg daily, sacubitril/valsartan 97/103mg twice daily, spironolactone 25mg daily, furosemide 40mg daily, apixaban 5mg twice daily, metformin 1000mg twice daily, and atorvastatin 80mg nightly. She is reporting NYHA class III symptoms. She underwent CRT-D placement and has regular device interrogations with appropriate biventricular pacing and no reported ICD therapies delivered since implant. Last hospitalization for decompensated heart failure was 9 months ago. On examination her blood pressure is 100/68 mm Hg, heart rate is 60 bpm, respiratory rate is 12/min, and oxygen saturation rate is 99% while breathing ambient air. She appears comfortable; there is no appreciable JVD. Her cardiovascular examination is notable for normal rate, regular rhythm, with no murmurs or gallops heard. Lungs are clear, abdomen is soft, and distal extremities are warm to touch.

Which one of the following is the next best step in the management of this patient?

- ☒ **A.** Initiate Dapagliflozin
- ☐ **B.** Decrease Sacubitril/Valsartan
- ☐ **C.** Refer for CPET
- ☐ **D.** Decrease metoprolol succinate to 100mg daily

Commentary References

Testing Point: The AHFTC specialist should recognize when to initiate SGLT2 inhibitor therapy as part of contemporary quadruple GDMT for HFrEF.

Answer: A. Initiate dapagliflozin The patient has compensated chronic HFrEF, and is on triple but not contemporary quadruple drug therapy. SGLT2 inhibitor therapy is appropriate for patients with heart failure, no matter what the ejection fraction, due to effectiveness in reducing hospitalizations, and the mortality benefit in HFrEF.

Answer B: Decrease Sacubitril/Valsartan The less experienced practitioner may select this because of a perception of low or marginal blood pressure, but in chronic HFrEF, this blood pressure is indicative of good afterload reduction and no dose adjustment should be prescribed in the absence of orthostatic symptoms.

Answer C: Refer for CPET. This patient is on good drug therapy, has not been recently hospitalized, and is not in need of advanced therapies at the present time. A CPET might be reasonable to establish functional capacity and prognosis in patients but the CPET itself will not change this patient's prognosis, while prescription of SGLT2 inhibitor will.

Answer D: Decrease metoprolol succinate to 100mg daily A less experienced practitioner may perceive the patient as having borderline resting heart rate, however there is no evidence of hypoperfusion and reduction of target doses of guidelines-based medical therapy should not be done without a compelling clinical indication as it may worsen HF outcomes.

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